

INTEGRATED MANAGEMENT OF CHILDHOOD ILLNESS

SICK CHILD AGE 2 MONTHS UP TO 5 YEARS

ASSESS AND CLASSIFY THE SICK CHILD

Assess, Classify and Identify Treatment

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WHO

MoH
Afghanistan

UNICEF

SICK YOUNG INFANT AGE BIRTH UP TO 2 MONTHS

ASSESS, CLASSIFY AND TREAT THE SICK YOUNG INFANT

Assess, Classify and Identify Treatment

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ASSESS AND CLASSIFY THE SICK CHILD AGE 2 MONTHS UP TO 5 YEARS

1) ASK THE MOTHER WHAT THE CHILD'S PROBLEMS ARE

Determine if this is an initial or follow-up visit for this problem—

If initial visit assess as follows; if a follow-up for this problem go to follow-up

2) CHECK FOR GENERAL DANGER SIGNS

ASK:

- Child is unable to drink or breastfeed
- Child vomits everything
- Child had convulsions

LOOK:

- Child is lethargic or unconscious
- Child is convulsing now

If YES

VERY SEVERE DISEASE

- ▶ **ASK** if cough, fever, diarrhea, sore throat & **LOOK** for severe malnutrition
- ▶ If convulsing give diazepam, Ampicillin & Gentamicin and manage the airway
- ▶ If cough or sore throat give Ampicillin & Gentamicin and refer
- ▶ If diarrhea, hydrate (Plan C), reassess and if child did not respond to hydration, refer
- ▶ If fever give quinine (if malaria endemic), Ampicillin, Gentamicin and Paracetamol (If fever 38.5 C or more)
- ▶ Treat the child to prevent low blood sugar
- ▶ If severe malnutrition give Vit A and refer

3) IF NO DANGER SIGNS ASSESS COUGH, FEVER, DIARRHOEA, THROAT/EAR PROBLEM:

Does the child have cough or difficult breathing? If yes...

ASK How many days?

LOOK if breathing fast
LOOK for chest indrawing
LISTEN for Stridor
Look and listen for wheeze

Stridor or chest indrawing

(If wheeze, go to treatment of wheezing)

YES

fast Breathing

(If wheeze, go to treatment of wheezing)

YES

No signs of pneumonia or very severe disease

(If wheeze, go to treatment of wheezing)

YES

SEVERE PNEUMONIA

- ▶ Give first dose Ampicillin & Gentamicin
- ▶ Treat wheeze if present treat with aerosolized salbutamol
- ▶ Refer **URGENTLY** to hospital.*

PNEUMONIA

- ▶ Give Cotrimoxazole for 5 days.
- ▶ Treat wheeze if present.
- ▶ Follow-up in 2 days
- ▶ Advise mother to return immediately if breathing faster or with more difficulty
- ▶ Counsel mother about safe home remedy

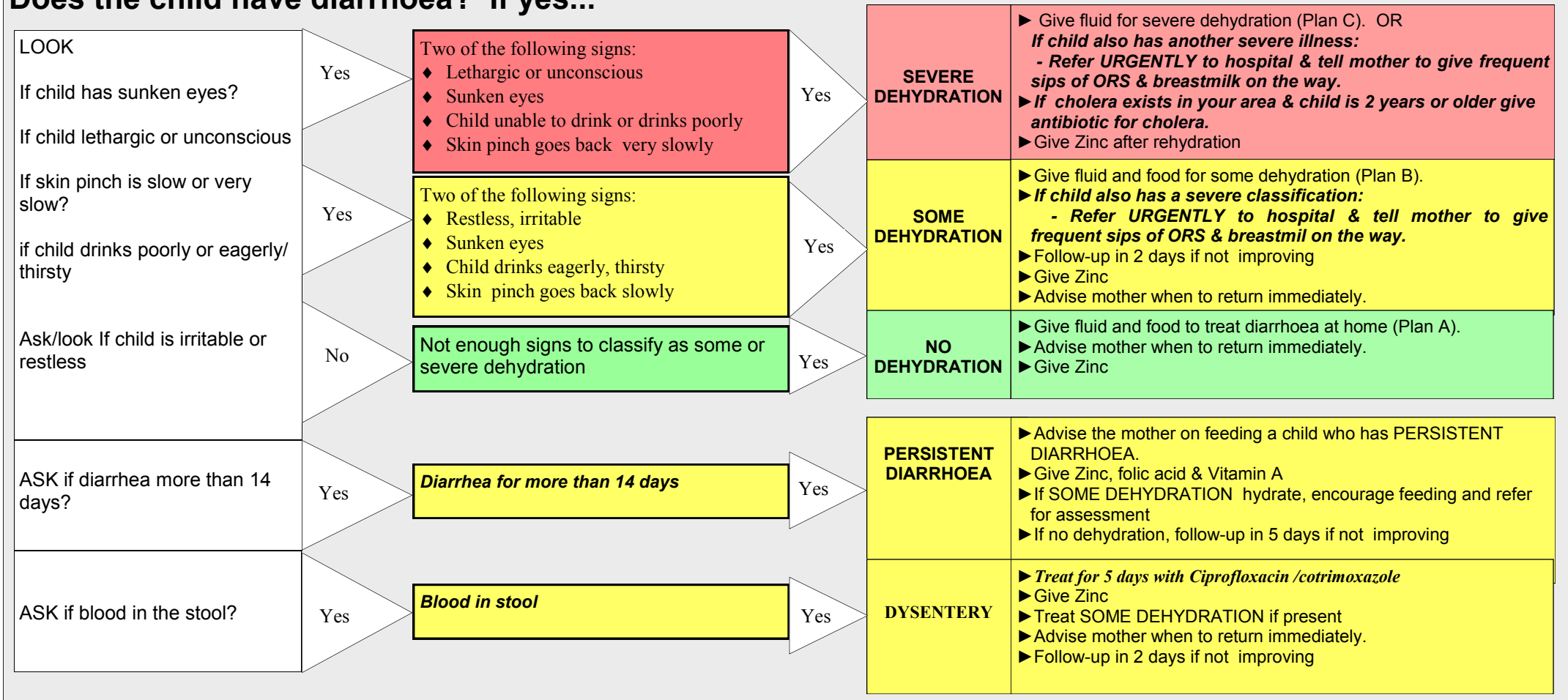
**NO PNEUMONIA:
COUGH OR
COLD**

- ▶ Treat wheeze if present
- ▶ If coughing more than 14 days refer for non-urgent assessment.
- ▶ Follow-up in 5 days if not improving
- ▶ Advise mother to return immediately
- ▶ Counsel mother about safe home remedy

Fast breathing: 2 to 12 months old 50 or more breaths each minute
12 to 59 months old 40 or more breaths each minute

*If referral is not possible, manage the child as described in *Integrated Management of Childhood Illness*, *Treat the Child*, Annex: Where Referral Is Not Possible, and WHO guidelines for inpatient care.

Does the child have diarrhoea? If yes...



Does the child have fever? If yes...

(by history or feels hot or temperature 37.5°C** or above)

Check stiff neck

Yes

**VERY SEVERE
FEBRILE
DISEASE**

- ▶ Give quinine for severe malaria (first dose) in malaria endemic area
- ▶ Give first dose of Ampicillin & Gentamicin
- ▶ Feed child with breastmilk or sugar water to prevent low blood sugar
- ▶ Give one dose of paracetamol in clinic for high fever (38.5°C or above).
- ▶ Refer **URGENTLY** to hospital.

ASK if child had measles in last 3 months

LOOK for measles (Generalized rash AND cough or runny nose or red eyes) if yes:

Yes

Clouded cornea or deep mouth ulcers

Yes

**SEVERE
COMPLICATED
MEASLES*****

- ▶ Give Vitamin A, if hasn't received does of vitamin A during last 3 months
- ▶ Give first dose of Ampicillin & Gentamicin.
- ▶ If clouding of the cornea or pus draining from the eye, apply tetracycline eye ointment.
- ▶ Refer **URGENTLY** to hospital.
- ▶ Advise the mother to continue feeding the child.

Yes

Eye infection or small mouth ulcers

Yes

**MEASLES WITH
EYE OR MOUTH
COMPLICATIONS*****

- ▶ Give Vitamin A if child has not received it within 3 months
- ▶ If pus draining from the eye, treat eye infection with tetracycline eye ointment.
- ▶ If mouth ulcers, treat with gentian violet.
- ▶ Advise the mother to continue feeding the child.
- ▶ Follow-up in 2 days.

No

Simple measles now or in last 3 months

Yes

Simple Measles now or within last 3 months

- ▶ Give Vitamin A if child has not received it within 3 months
- ▶ Counsel the mother to continue feeding the child
- ▶ Follow-up in 2 days if no improvement

LOOK for
- clouded cornea
- mouth ulcers
- eye infection

DECIDE if high or low malaria risk

LOOK for other source of fever:

- Sore throat, ear infection, ARI, diarrhea or any other cause of fever

None

High or Low Malaria Risk

Yes

MALARIA

- ▶ Treat with oral antimalarial (Chloroquine + Sulphadoxine Pyremethamin)
- ▶ Give one dose of paracetamol in clinic for high fever (38.5°C or above).
- ▶ Advise mother when to return immediately.
- ▶ Follow-up in 2 days if fever persists.

Yes

High Malaria

Yes

**MALARIA AND
OTHER INFECTION**

- ▶ Treat with oral antimalarial (Chloroquine + Sulphadoxine Pyremethamin & if other infection is present, give appropriate antibiotic
- ▶ Give one dose of paracetamol in clinic for high fever (38.5°C or above).
- ▶ Advise mother when to return immediately.

Low Malaria

Yes

**FEVER -
OTHER INFECTION
MALARIA
UNLIKELY**

- ▶ Give one dose of paracetamol in clinic for high fever (38.5°C or above) & if other infection is present, give appropriate antibiotic
- ▶ Advise mother when to return immediately.
- ▶ Follow-up in 2 days if fever persists.
- ▶ If fever is present every day for more than 7 days, refer for assessment.

** These temperatures are based on axillary temperature. Rectal temperature readings are approximately 0.5°C higher.

*** Other important complications of measles - pneumonia, stridor, diarrhoea, ear infection, and malnutrition - are classified in other tables.

4) Does the child have an ear problem? If yes....

Ask if there is ear pain Look for discharge from ear - For how long? FEEL for tender spot behind ear	Tender swelling behind ear	Yes	MASTOIDITIS	▶ Give first dose of an antibiotic (Ampicillin + Gentamicin). ▶ Give first dose of paracetamol for pain. ▶ Refer URGENTLY to hospital.
	♦ Pus draining from ear less than 14 days OR ♦ ear pain	Yes	ACUTE EAR INFECTION	▶ Give Cotrimoxazole for 5 days . ▶ Give paracetamol for pain. ▶ Dry the ear by wicking ▶ Follow-up in 5 days.
	Pus draining from ear longer than 14 days	Yes	CHRONIC EAR INFECTION	▶ Dry the ear by wicking. ▶ Follow-up in 5 days.

5) Does the child have a throat problem? If yes...

ASK if there is throat pain LOOK for red throat, white exudates on tonsils FEEL for swollen lymph nodes	Throat pain and 2 of the following: -Red throat OR -Exudates on tonsils/throat OR -Large, tender neck lymph nodes	Yes	STREPTOCOCCAL SORETHROAT	▶ Give Amoxicillin for 5 days ▶ Soothe throat with safe remedy ▶ Give paracetamol for pain. ▶ Advise when to return immediately ▶ Follow-up in 5 days if no improvement
	Throat pain and 1 or none of the symptoms above	Yes	VIRAL SORETHROAT	▶ Soothe throat with safe remedy ▶ Give paracetamol for pain. ▶ Advise when to return immediately ▶ Follow-up in 5 days if no improvement

6) Check for malnutrition

LOOK FOR • Visible wasting • Oedema of both feet • Determine weight for age	• Visible severe wasting OR • Oedema of both feet.	Yes	SEVERE MALNUTRITION	► <i>Give Vitamin A.</i> ► <i>Treat child to prevent low blood sugar</i> ► <i>Refer URGENTLY to hospital.</i>
	• Very Low weight for age	Yes	VERY LOW WEIGHT	► Assess the child's feeding and counsel the mother on feeding - If feeding problem, follow-up in 5 days ► Advise mother when to return immediately ► Follow-up in 30 days ► Give mebendazole if child is 1 year or older and has had no mebendazole within last 6 months. ► If Very low weight for age, follow-up in 30 days.
	• Not very low weight for age and no other signs of malnutrition.	Yes	NOT VERY LOW WEIGHT	► If child is less than 2 years old, assess the child's feeding and counsel the mother on feeding - If feeding problem, follow-up in 5 days.

7) Check for anemia

LOOK FOR • Palmar pallor Is it some or severe?	• Severe palmar pallor	Yes	SEVERE ANAEMIA	► <i>Treat the child to prevent low blood sugar</i> ► <i>Refer URGENTLY to hospital</i>
	• Some palmar pallor	Yes	ANAEMIA	► Assess the child's feeding and counsel the mother on feeding - If feeding problem, follow-up in 5 days. ► Give Iron ► <i>Give oral antimalarial if high malaria risk</i> ► <i>Deworm if child has not received mebendazole within the last 6 months (only if child is 1 year or older)</i> ► Advise mother when to return immediately ► Follow-up in 14 days
	• No palmar pallor	Yes	NO ANEMIA	► If child is less than 2 years old, assess the child's feeding and counsel the mother on feeding - If feeding problem, follow-up in 5 days.

8) Check immunizations (for 2-23 months), Vitamin A and de-worming status (for all 2-59 months)

If age 2-23 months: IMMUNIZATION SCHEDULE:	AGE	VACCINE	VITAMIN A SUPPLEMENTATION STATUS: • Is child age 6 months or older? • Has child received vitamin A in the previous 6 months?	DEWORMING STATUS For a child 1 year or older who has not received a deworming drug in the last 6 months give Mebendazole one dose.
	Birth 6 weeks 10 weeks 14 weeks 9 months 18 months	BCG Penta-1 Penta-2 Penta-3 Measles-1 Measles-2		
		OPV-0 OPV-1 OPV-2 OPV-3 OPV-4		

9) Assess other problems



TREAT THE CHILD

CARRY OUT THE TREATMENT STEPS IDENTIFIED ON THE ASSESS AND CLASSIFY CHART



TEACH THE MOTHER TO GIVE ORAL DRUGS AT HOME

Follow the instructions below for every oral drug to be given at home. Also follow the instructions listed with each drug's dosage table.

- Determine the appropriate drugs and dosage for the child's age or weight.
- Tell the mother the reason for giving the drug to the child.
- Explain carefully how to give the drug
- Check the mother's understanding before she leaves the clinic.
- Explain that all the oral drug tablets or syrups must be used to finish the course of treatment, even if the child gets better.

The health worker dispensing medication should:

- Demonstrate how to measure a dose
- Watch the mother practise measuring a dose by herself
- Ask the mother to give the first dose to her child
- Label and package the drug
- If more than one drug is given, collect, count and package each drug separately

► Give an Appropriate Oral Antibiotic

► FOR PNEUMONIA, ACUTE EAR INFECTION:

FIRST-LINE ANTIBIOTIC: COTRIMOXAZOLE
SECOND-LINE ANTIBIOTIC: AMOXICILLIN

► FOR STREP THROAT:

AMOXICILLIN
ERYTHROMYCIN

	COTRIMOXAZOLE (trimethoprim + sulphamethoxazole) ↳ Give two times daily for 5 days	AMOXICILLIN ↳ Give three times daily for 5 days	Erythromycin Give four times daily for 5 days
AGE or WEIGHT	ADULT TABLET 80 mg trimethoprim + 400 mg sulphamethoxazole	TABLET 250 mg	TABLET 250 mg
2 months up to 4 months (4 - <6 kg)	1/2	1/2	1/4
4 months up to 12 months (6 - <10 kg)	1	1	1/2
12 months up to 5 years (10 - 19 kg)	1	1	1

► FOR DYSENTERY:

Give antibiotic recommended for Shigella for 5 days.

FIRST-LINE ANTIBIOTIC FOR SHIGELLA:

SECOND-LINE ANTIBIOTIC FOR SHIGELLA:

CIPROFLOXACIN OR COTRIMOXAZOLE

METRONIDAZOLE

	CIPROFLOXACIN Give two times daily for 5 days	COTRIMOXAZOLE (trimethoprim + sulphamethoxazole) ↳ Give two times daily for 5 days	METRONIDAZOLE ↳ Give three times daily for 5 days
AGE or WEIGHT	TABLET (250 mg)		TABLET (200mg)
2 months up to <6 months (4 - <6 kg)	1/4	See above	1/4
6 months up to <12 months (6kg—<10kg)	1/2		
12 months up to <3 years (10 - <14 kg)	1		1/2
3 years to 5 years (14-19 kg)	1		1

► FOR CHOLERA:

Give antibiotic recommended for Cholera for 3 days. Follow national recommendation if antibiogram available

	COTRIMOXAZOLE (trimethoprim + sulphamethoxazole) ↳ Give two times daily for 3 days	ERYTHROMYCIN ↳ Give four times daily for 3 days
AGE or WEIGHT		TABLET 250 mg
2 months up to 4 months (4 - <6 kg)	See doses above	1/4
4 months up to 12 months (6 - <10 kg)		1/2
12 months up to 5 years (10 - 19 kg)		1

ANTIBIOTICS

TREAT

TEACH THE MOTHER TO GIVE ORAL DRUGS AT HOME

Follow the instructions below for every oral drug to be given at home.
Also follow the instructions listed with each drug's dosage table.

► *Give an Oral Antimalarial*

FIRST-LINE ANTIMALARIAL: CHLOROQUINE +SULFADOXINE PYREMETHAMINE
SECOND-LINE ANTIMALARIAL: QUININE

► FOR CHLOROQUINE:

- Explain to the mother that she should watch her child carefully for 30 minutes after giving a dose of chloroquine. If the child vomits within 30 minutes, she should repeat the dose and return to the clinic for additional tablets.
- Explain that itching is a possible side effect of the drug, but is not dangerous.

► FOR SULFADOXINE + PYRIMETHAMINE: Give single dose in clinic.

CHLOROQUINE							SULFADOXINE + PYRIMETHAMINE
► Give for 3 days							► Give single dose in clinic
AGE or WEIGHT	TABLET (150 mg base)			SYRUP (50 mg base per 5 ml)			TABLET (500 mg sulfadoxine +25 mg pyrimethamine)
	DAY 1	DAY 2	DAY 3	DAY 1	DAY 2	DAY 3	
2 months up to 12 months (4 - <10 kg)	1/2	1/2	1/2	7.5 ml	7.5 ml	5.0 ml	1/2
12 months up to 3 years (10 - <14 kg)	1	1	1/2	15.0 ml	15.0 ml	5.0 ml	1
3 years up to 5 years (14 - 19 kg)	1 1/2	1 1/2	1/2				1

► *Paracetamol for High Fever (> 38.5°C) or Ear Pain*

PARACETAMOL			► Give
AGE or WEIGHT	TABLET (100 mg)	TABLET (500 mg)	
2 months up to 3 years (4 - <14 kg)	1	1/4	
3 years up to 5 years (14 - <19 kg)	1 1/2	1/2	

paracetamol every 6 hours until high fever or ear pain is gone.

► *Oral Salbutamol*

- Give Salbutamol tablets three times daily for 5 days.

AGE or WEIGHT	SALBUTAMOL TABLET (2 mg)	SALBUTAMOL TABLET (4 mg)
2 months up to 6 months (4 - <6 kg)	NONE	NONE
6 months up to 12 months (6 - <10 kg)	1/2 TABLET	1/4 TABLET
12 months up to 5 years (10 - <19 kg)	1 TABLET	1/2 TABLET

► *Vitamin A*

- For measles give three doses.
 - Give first dose in clinic.
 - Give mother two doses to give at home: one next day, one on day 14.
- For severe malnutrition, give one dose before referral to hospital.
- For vitamin supplementation for a child 6 months or older who receive no vitamin A within the last 6 months, give first dose in clinic
- For persistent diarrhea, give one dose

AGE	VITAMIN A CAPSULES		
	200 000 IU	100 000 IU	50 000 IU
6 months up to 12 months	1/2 capsule	1 capsule	2 capsules
12 months up to 5 years	1 capsule	2 capsules	4 capsules

► *Iron*

- Give one dose daily for 14 days.

AGE or WEIGHT	IRON/FOLATE TABLET Ferrous sulfate 200 mg + 250 mcg Folate (60 mg elemental iron)
2 months up to 12 months (4 - <10 kg)	1/4 tablet
12 months up to 5 years (10 - <19 kg)	1/2 tablet

► *Zinc Tablets*

- For Diarrhea give one dose daily for 10 days.

AGE	ZINC TAB 20mg
Up to 6 months	0.5 tablet
6 months up to 5 years	1 tablet

► *Folic Acid*

- For persistent diarrhoea, give 1 tablet for 14 days
1 mg/day

► *Mebendazole*

- Give one dose of mebendazole if
 - child is 1 year of age or older, and
 - the child has had none in the last 6 months
 - 250 mg if the child is less than 2 years
 - 500 mg if the child is 2 years of age or older

TEACH THE MOTHER TO TREAT LOCAL INFECTIONS AT HOME

- ▶ Tell the mother the importance of the treatment
- ▶ Describe the treatment steps listed in the appropriate box.
- ▶ Check the mother's understanding before she leaves the clinic.

▶ *Treat Eye Infection with Tetracycline Eye Ointment*

- ▶ Clean both eyes 3 times daily.
 - Wash hands.
 - Ask child to close the eye.
 - Use clean cloth and water to gently wipe away pus.
- ▶ Then apply tetracycline eye ointment in both eyes 3 times daily.
 - Ask the child to look up.
 - Squirt a small amount of ointment on the inside of the lower lid.
 - Wash hands again.
- ▶ Treat until redness is gone.
- ▶ Do not use other eye ointments or drops, or put anything else in the eye.

▶ *Dry the Ear by Wicking*

- ▶ Dry the ear at least 3 times daily.
 - Roll clean absorbent cloth into a wick.
 - Place the wick in the child's ear.
 - Remove the wick when wet.
 - Replace the wick with a clean one and repeat these steps until the ear is dry.

▶ *Treat Mouth Ulcers with Gentian Violet*

- ▶ Treat the mouth ulcers twice daily
 - Wash hands
 - Wash the child's mouth with clean soft, wet cloth with salt wrapped around the finger
 - Paint the mouth with half-strength gentian violet
 - Wash hands again

▶ *Soothe the Throat & Cough*

- Safe remedies to recommend as needed:
 - Breastmilk for exclusively breastfed infant.
 - warm soup, sweet tea, warm water, milk with bee honey, khatmi, zard choba
- Harmful remedies to discourage:
 - All cough syrups and expectorants,
 - All nasal drops or ear drops
 - Rubbing and tightening the chest with plastic
 - Tariak

GIVE THESE TREATMENTS ONLY IN CLINIC

- Explain the reason for the medication
- Determine the dose appropriate for the child's weight (or age).
- Use a sterile needle and sterile syringe. Measure the dose accurately.
- If child cannot be referred, follow the instructions provided.

► **Diazepam for the convulsing child**

Manage the Airway

- Turn the child on his/her side to avoid aspiration.
- Do not insert anything in the mouth.
- If the lips and tongue are blue, open the mouth and make sure the airway is clear.
- If necessary, remove secretions from the throat through a catheter inserted through the nose.

Give Diazepam Rectally

- Draw up the dose from an ampoule of diazepam into a tuberculin syringe. Then remove the needle.
- Insert the syringe 4 to 5 cm into the rectum and inject the diazepam solution.
- Hold the buttocks together for a few minutes

AGE or WEIGHT	DIAZEPAM GIVEN RECTALLY 10 mg/ 2 ml Solution
1 months up to 4 months (3 - < 6 kg)	0.5 ml
4 months up to 12 months (6 - < 10 kg)	1.0 ml
12 months up to 3 years (10 - < 14 kg)	1.25 ml
3 years up to 5 years (14 - 19 kg)	1.5 ml

For High Fever

- Sponge the child with tepid water.
- Give antipyretic

Treat the Child to prevent Low Blood sugar

► **Intramuscular Antibiotic**

FOR CHILDREN BEING REFERRED URGENTLY WHO CANNOT TAKE AN ORAL ANTIBIOTIC:

- Give first dose of intramuscular Ampicillin and Gentamycin and refer child urgently to hospital.

IF REFERRAL IS NOT POSSIBLE:

- Repeat the Ampicillin every 6 hours and Gentamycin injection every 12 hours until the child has improved.
- Then change to an appropriate oral antibiotic to complete 10 days of treatment.

AGE or WEIGHT	AMPICILLIN (4 times daily)	GENTAMICIN (twice daily)
	Dose: 50 mg per kg PER DOSE Add 4.5 ml sterile water to vial containing 500 mg = 100MG/ML	Dose: 3.75mg per kg PER DOSE Add 2 ml sterile water to vial containing 20 mg = 10MG/ML
2 months up to 4 months (4 - < 6 kg)	2.0 ml = 200 mg	1.5 ml = 15 mg
4 months up to 9 months (6 - < 8 kg)	3.0 ml = 300 mg	2.25 ml = 22.5 mg
9 months up to 12 months (8 - < 10 kg)	4.0 ml = 400 mg	3.0 ml = 30 mg
12 months up to 3 years (10 - < 14 kg)	5.0 ml = 500 mg	3.75 ml = 37.5 mg
3 years up to 5 years (14 - 19 kg)	7.0 ml = 700 mg	5 ml = 50 mg

► **Quinine for Severe Malaria**

FOR CHILDREN BEING REFERRED WITH VERY SEVERE FEBRILE DISEASE:

- Check which quinine formulation is available in your clinic.
- Give first dose of intramuscular quinine and refer child urgently to hospital.

AGE or WEIGHT	INTRAMUSCULAR QUININE	
	150 mg/ml* (in 2 ml ampoules)	300 mg/ml* (in 2 ml ampoules)
2 months up to 4 months (4 - < 6 kg)	0.4 ml	0.2 ml
4 months up to 12 months (6 - < 10 kg)	0.6 ml	0.3 ml
12 months up to 2 years (10 - < 12 kg)	0.8 ml	0.4 ml
2 years up to 3 years (12 - < 14 kg)	1.0 ml	0.5 ml
3 years up to 5 years (14 - 19 kg)	1.2 ml	0.6 ml

* quinine salt

IF REFERRAL IS NOT POSSIBLE:

- Give first dose of intramuscular quinine.
- The child should remain lying down for one hour.
- Repeat the quinine injection at 4 and 8 hours later, and then every 12 hours until the child is able to take an oral antimalarial. Do not continue quinine injections for more than 1 week.
- If low risk of malaria, do not give quinine to a child less than 4 months of age.

➤ *Treat Wheezing*

- Children with wheezing and
GENERAL DANGER SIGN
OR STRIDOR or chest indrawing → Give one dose of pre referral antibiotic and one
dose of rapid acting bronchodilator and refer immediately
STRIDOR or chest indrawing
- Children with wheezing and
fast breathing but NO GENERAL
DANGER SIGN, NO STRIDOR → Treat for PNEUMONIA &
give oral salbutamol for 5 days, If child is 6 or more
than 6 months old
- Children with ONLY wheezing → Treat for NO PNEUMONIA: COUGH OR COLD
Give oral salbutamol for 5 days , If child is 6 or more
than 6 months old

AGE or WEIGHT	RAPID ACTING BRONCHODILATOR	ORAL SALBUTAMOL	ORAL SALBUTAMOL
	Nebulized Salbutamol (5mg/ml)	TABLET (2 mg)	TABLET (4 mg)
2 months up to 6 months (4 - <7 kg)	0.25ml plus 2.0ml sterile water	NONE	NONE
6 months up to 12 months (7 - <10 kg)	0.5ml Salbutamol plus 2.0 ml sterile water	1/2 TABLET	1/4 TABLET
12 months up to 5 years (10 - <19 kg)	0.5ml Salbutamol plus 2.0 ml sterile water	1 TABLET	1/2 TABLET

▶ *Prevent Low Blood Sugar*

- ▶ **Best option:** Counsel the mother to breastfeed the child if the child is able
- ▶ **Second Best:** Give sugar water if the child is unable to breastfeed
**Make sugar water by dissolving 4 level teaspoons of sugar
(20 grams) in a 200-ml cup of clean water**
- ▶ **If necessary:** *If the child is not able to swallow* give 50 ml of milk or sugar water by
nasogastric tube

GIVE EXTRA FLUID FOR DIARRHOEA AND CONTINUE FEEDING

(See **FOOD** advice on **COUNSEL THE MOTHER** chart)

► Plan A: Treat Diarrhoea at Home

Consider the following four principles:

1. GIVE EXTRA FLUID UNTIL DIARRHEA STOPS

► TELL THE MOTHER:

- Breastfeed as much & as often as the child will take
- If child exclusively breastfeeds or is less than 6 months old, give ORS or clean water after breastmilk as much as child will take
- If the child is not exclusively breastfed, give ORS solution, soup, rice water, yoghurt drinks, or clean water.
- Give frequent small sips from a cup.
- If the child vomits, wait 10 minutes. Then continue, but more slowly.
- Continue giving extra fluid until the diarrhoea stops.
- Offer the child a drink after each loose stool

► TEACH THE MOTHER HOW TO MIX AND GIVE ORS & GIVE HER 2 PACKETS OF ORS TO USE AT HOME. Tell her to give as much as the child will take after breastfeeding

2. CONTINUE FEEDING

3. GIVE ZINC

4. WHEN TO RETURN

► Plan B: Treat Some Dehydration with ORS

Hydrate child with ORS while in clinic over 4-hour period

► AMOUNT OF ORS TO GIVE DURING FIRST 4 HOURS.

AGE*	Up to 4 months	4 months up to 12 months	12 months up to 2 years	2 years up to 5 years
WEIGHT	< 6 kg	6 - < 10 kg	10 - < 12 kg	12 - 19 kg
In ml	200 - 400	400 - 700	700 - 900	900 - 1400

* Use the child's age only when you do not know the weight. The approximate amount of ORS required (in ml) can also be calculated by multiplying the child's weight (in kg) times 75.

- If the child wants more ORS than shown, give more.
- For infants under 6 months who are not breastfed, also give 100-200 ml clean water during this period.

► SHOW THE MOTHER HOW TO GIVE ORS SOLUTION.

- Give frequent small sips from a cup.
- If the child vomits, wait 10 minutes. Then continue, but more slowly.
- Continue breastfeeding whenever the child wants.

► AFTER 4 HOURS:

- Reassess the child and classify the child for dehydration.
- Select Plan A, B or C according to the new classification.
- Begin feeding the child in clinic.

► IF THE MOTHER MUST LEAVE BEFORE COMPLETING TREATMENT:

- Show her how to prepare ORS solution at home.
- Show her how much ORS to give to finish 4-hour treatment at home.
- Give her enough ORS packets to complete rehydration. Also give her 2 packets as recommended in Plan A and explain Plan A.

1. GIVE EXTRA FLUID

2. CONTINUE FEEDING

3. GIVE ZINC

4. WHEN TO RETURN



See Plan A for recommended fluids
and
See **COUNSEL THE MOTHER** chart

GIVE EXTRA FLUID FOR DIARRHOEA AND CONTINUE FEEDING

(See **FOOD** advice on **COUNSEL THE MOTHER** chart)

► **Plan C: Treat Severe Dehydration Quickly**

► FOLLOW THE ARROWS. IF ANSWER IS "YES", GO ACROSS. IF "NO", GO DOWN.

START HERE

Can you give intravenous (IV) fluid immediately?

YES

- Start IV fluid immediately. If the child can drink, give ORS by mouth while the drip is set up. Give 100 ml/kg Ringer's Lactate Solution (or, if not available, normal saline), divided as follows:
- * Repeat once if radial pulse is still very weak or not detectable.

AGE	First give 30 ml/kg in:	Then give 70 ml/kg in:
Infants (under 12 months)	1 hour*	5 hours
Children (12 months up to 5 years)	30 minutes*	2 1/2 hours

- Reassess the child every 1-2 hours. If hydration status is not improving, give the IV drip more rapidly.
- Also give ORS (about 5 ml/kg/hour) as soon as the child can drink: usually after 3-4 hours (infants) or 1-2 hours (children).
- Reassess an infant after 6 hours and a child after 3 hours. Classify dehydration. Then choose the appropriate plan (A, B, or C) to continue treatment.

NO

Is IV treatment available nearby (within 30 minutes)?

YES

- Refer URGENTLY to hospital for IV treatment.
- If the child can drink, provide the mother with ORS solution and show her how to give frequent sips during the trip.

NO

Are you trained to use a naso-gastric (NG) tube for rehydration?

YES

- Start rehydration by tube (or mouth) with ORS solution: give 20 ml/kg/hour for 6 hours (total of 120 ml/kg).
- Reassess the child every 1-2 hours:
 - If there is repeated vomiting or increasing abdominal distension, give the fluid more slowly.
 - If hydration status is not improving after 3 hours, send the child for IV therapy.
- After 6 hours, reassess the child. Classify dehydration. Then choose the appropriate plan (A, B, or C) to continue treatment.

NO

Can the child drink?

NO

Refer URGENTLY to hospital for IV or NG treatment

NOTE:

- If possible, observe the child at least 6 hours after rehydration to be sure the mother can maintain hydration giving the child ORS solution by mouth.

GIVE FOLLOW-UP CARE

The purpose of follow-up care is to assess

- ◆ If the treatment given was effective
- ◆ Whether the child develops a new illness and requires assessment and treatment for this new problem—use the **ASSESS AND CLASSIFY** chart
- ◆ Whether the child needs referral. Use the boxes that match the child's previous diagnoses.

► PNEUMONIA

After 2 days:

- Check the child for general danger signs
- And reassess cough or difficulty breathing by using the ASSESS & CLASSIFY Chart:

- ◆ Ask is the child breathing slower
- ◆ Is there less fever
- ◆ Is the child eating better

-Follow the treatment plans below:

- If **chest indrawing or a general danger sign**, give a dose of second-line antibiotic or intramuscular ampicillin & gentamycin. Then refer URGENTLY to hospital.
- If **breathing rate, fever and eating are the same**, change to the second-line antibiotic and advise the mother to return in 2 days. Refer if this child had measles within the last 3 months.
- If **child has slower breathing, less fever, or is eating better**, complete the 5 days of antibiotic.

► PERSISTENT DIARRHOEA

After 5 days:

Ask:

- Has the diarrhoea stopped?
- How many loose stools is the child having each day?

Treatment:

- If **the diarrhoea has not stopped (child is still having 3 or more loose stools per day)**, do a full reassessment of the child. Give any treatment needed. Then refer to hospital.
- If **the diarrhoea has stopped (child having less than 3 loose stools per day)**, tell the mother to follow the usual feeding recommendations for the child's age.

► DYSENTERY

After 2 days:

Assess the child for diarrhoea. > See ASSESS & CLASSIFY chart.

Ask:

- Are there fewer stools?
- Is there less blood in the stool?
- Is there less fever?
- Is there less abdominal pain?
- Is the child eating better?

Treatment:

- If the child is **dehydrated**, treat dehydration.
- If **number of stools, amount of blood in stools, fever, abdominal pain, or eating is the same or worse:**

Change to second-line oral antibiotic recommended for Shigella in your area. Give it for 5 days. Advise the mother to return in 2 days.

Exception: refer to hospital if the child is

- is less than 12 months old, or
- was dehydrated on the first visit, or
- had measles within the last 3 months

- If **fewer stools, less blood in the stools, less fever, less abdominal pain, and eating better**, continue giving the same antibiotic until finished.

GIVE FOLLOW-UP CARE

The purpose of follow-up care is to assess

- ◆ If the treatment given was effective
- ◆ Whether the child develops a new illness and requires assessment and treatment for this new problem—use the **ASSESS AND CLASSIFY** chart
- ◆ Whether the child needs referral. Use the boxes that match the child's previous diagnoses.

► MALARIA (Low or High Malaria Risk)

If fever persists after 2 days, or returns within 14 days:

Do a full reassessment of the child looking for other causes of the problem. > See **ASSESS & CLASSIFY** chart.

Treatment:

- If the child has **any general danger sign or stiff neck**, treat as VERY SEVERE FEBRILE DISEASE.
- If the child has any **cause of fever other than malaria**, provide treatment.
- If **malaria is the only apparent cause of fever**:
 - Treat with the second-line oral antimalarial. (If no second-line antimalarial is available, refer to hospital.) Advise the mother to return again in 2 days if the fever persists.
 - If fever has been present for 7 days, refer for assessment.

► FEVER-MALARIA UNLIKELY (Low Malaria Risk)

If fever persists after 2 days:

Do a full reassessment of the child looking for other causes of the problem. > See **ASSESS & CLASSIFY** chart.

Assess for other causes of fever.

Treatment:

- If the child has **any general danger sign**, treat as VERY SEVERE DISEASE
- If the child has **stiff neck**, treat as VERY SEVERE FEBRILE DISEASE.
- If the child has any **cause of fever other than malaria**, provide treatment.
- If **malaria is the only apparent cause of fever**:
 - Treat with the first-line oral antimalarial. Advise the mother to return again in 2 days if the fever persists.
 - If fever has been present for 7 days, refer for assessment.

► MEASLES WITH EYE OR MOUTH COMPLICATIONS

After 2 days:

Look for red eyes and pus draining from the eyes.

Look at mouth ulcers.

Smell the mouth.

Treatment for Eye Infection:

- If **pus is draining from the eye**, ask the mother to describe how she has treated the eye infection. If treatment has been correct, refer to hospital. If treatment has not been correct, teach mother correct treatment.
- If **the pus is gone but redness remains**, continue the treatment.
- If **no pus or redness**, stop the treatment.

Treatment for Mouth Ulcers:

- If **mouth ulcers are worse, or there is a very foul smell from the mouth**, refer to hospital.
- If **mouth ulcers are the same or better**, continue using half-strength gentian violet for a total of 5 days.
- **Advise the mother to continue feeding the child**

GIVE FOLLOW-UP CARE

- **Care for the child who returns for follow-up using all the boxes that match the child's previous classifications.**
- **If the child has any new problem, assess, classify and treat the new problem as on the *ASSESS AND CLASSIFY* chart.**

► EAR INFECTION

After 5 days:

Reassess for ear problem. > See *ASSESS & CLASSIFY* chart.
Measure the child's temperature.

Treatment:

- If there is **tender swelling behind the ear or high fever (38.5°C or above)**, refer URGENTLY to hospital.
- **Acute ear infection:** if **ear pain or discharge** persists, treat with second line antibiotic for five more days. Continue wicking to dry the ear. Follow-up in 5 days.
- **Chronic ear infection:** Check that the mother is wicking the ear correctly. Encourage her to continue.

Note: If **no ear pain or discharge**, praise the mother for her careful treatment. If she has not yet finished the 5 days of antibiotic, tell her to use all of it before stopping.

► FEEDING PROBLEM

After 5 days:

Reassess feeding. > See *questions at the top of the COUNSEL* chart.
Ask about any feeding problems found on the initial visit.

- Counsel the mother about any new or continuing feeding problems. If you counsel the mother to make significant changes in feeding, ask her to bring the child back again.
- If the child is low weight for age, reweigh the child 30 days after the initial visit to measure the child's weight gain.

► PALLOR

After 14 days:

- Give iron. Advise mother to return in 14 days for more iron.
- Continue giving iron every 14 days for 2 months.
- If the child has palmar pallor after 2 months, refer for assessment.

► VERY LOW WEIGHT

After 30 days:

Weigh the child and determine if the child is still very low weight for age.
Reassess feeding. > See *questions at the top of the COUNSEL* chart.

Treatment:

- If the child is **no longer very low weight for age**, praise the mother and encourage her to continue.
- If the child is still **very low weight for age**, counsel the mother about feeding problem found. Ask the mother to return again in one month. Continue to see the child monthly until the child is feeding well and gaining weight regularly or is no longer very low weight for age.

Exception:

If you do not think that feeding will improve, or if the child has **lost weight**, refer the child.

**IF ANY MORE FOLLOW-UP VISITS ARE NEEDED
BASED ON THE INITIAL VISIT OR THIS VISIT,
ADVISE THE MOTHER OF THE
NEXT FOLLOW-UP VISIT**

•
**ALSO, ADVISE THE MOTHER
WHEN TO RETURN IMMEDIATELY.
(SEE COUNSEL CHART.)**



COUNSEL THE MOTHER



FOOD

► ***Assess the Child's Feeding***

Ask questions about the child's usual feeding and feeding during this illness. Compare the mother's answers to the ***Feeding Recommendations*** for the child's age in the box below.

ASK -

- Do you breastfeed your child?
 - How many times during the day?
 - Do you also breastfeed during the night?
- Does the child take any other food or fluids?
 - What food or fluids?
 - How many times per day?
 - What do you use to feed the child?
 - If very low weight for age: How large are servings? Does the child receive his own serving? Who feeds the child and how?
- During this illness, has the child's feeding changed? If yes, how?

► Feeding Recommendations During Sickness and Health

Up to 6 Months of Age



- Breastfeed as often as the child wants, day and night, at least 8 times in 24 hours.
- Do not give other foods or fluids such as :
 - zouf/bartang,
 - sakodana,
 - flous,
 - tea,
 - water,
 - sugar water,
 - cow's milk.
- Do not use bottle or pacifiers

6 Months up to 9 Months



- Breastfeed as often as the child wants, day or night, at least 8 times in 24 hours.
- add complementary foods 3 times per day after breastfeeding:
 - ✓ **Shola:** half cup of rice flour, half cup of cow's or goat's milk & 1 cup of water. Boil & cook slowly for 15 minutes. Add 1 teaspoon oil or *ghee*
 - ✓ **Lank bread porridge:** mix half cup of wheat flour, half cup of cow's or goat's milk, 1 and half teaspoon of sugar, 1 teaspoon of oil or *ghee*. Bake the mixture. Ground the *lank bread* into powder. Boil 1 cup of water with half cup of milk. Add the powdered lank bread and stir for 2 minutes)
 - ✓ **Mash porridge** (Cook for 20 minutes 1 and half tablespoons of *mash* or *paiti* . Once cooked, mash into porridge. Add 1/2 cup of wheat or rice or corn flour, 1 teaspoons of oil or *ghee* and half cup of water. Cook the mixture for 10 minutes).
 - ✓ **Superflour** (Toast separately, one cup of rice grains, one cup of corn grains and or cup of chickpeas or mash or lentils. Ground the grains separately. Mix half cup of toasted rice flour, half cup of toasted corn flour and half cup of toasted pulses flour (superflour). Mix half cup of the superflour and 1 and half cup of water and cook for 15 minutes. Add 1 teaspoon of oil or *ghee* at the end).

9 Months up to 12 Months

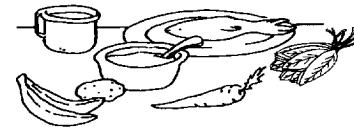


- Breastfeed as often as the child wants.
- Add complementary foods
- Give adequate servings of:
 - ✓ *Shola* or
 - ✓ *Lank bread* porridge or
 - ✓ *Mash* porridge or
 - ✓ *Superflour* porridge.
- 2 times per day if breastfed;
- 4 times per day if not breastfed.
- Give once daily, family foods
- Give once daily, "finger foods" such as half banana, grated apple or bread strips.

12 Months up to 2 Years



- Breastfeed as often as the child wants.
- Give once adequate servings of:
 - ✓ *Shola* or
 - ✓ *Lank bread* porridge or
 - ✓ *Mash* porridge or
 - ✓ *Superflour* porridge or
- Give family food 5 times per day.
- Give once daily, "finger foods" or snacks as 1/2 cup of yoghurt (*mast*) with 1 teaspoon of honey, small piece of cheese (*Paneer*), or a handful of almonds or dried fruit such as raisins.



2 Years and Older



- Give family foods at 3 meals each day such as:
 - ✓ Three tablespoons of pulses (*mash*, *dal*, *liti*, chickpeas, beans) or
 - ✓ One small piece of meat or fish or 1 egg + 3 tablespoons of (spinach, okra) or orange or (pumpkin, carrots) + 1 teaspoon of oil. With one cup of rice or boiled potato or *aash*.
- Also, twice daily, give "finger foods" such as half banana, grated apple or bread strips.



Feeding Recommendations For a Child Who Has PERSISTENT DIARRHOEA

- If breastfeeding, give more frequent, longer breastfeeds, day and night. Do not give other milk.
- If taking other milk:
 - replace with increased breastfeeding OR
 - replace with fermented milk products, such as yoghurt OR
 - replace half the milk with nutrient-rich semisolid food like cereals, pulses, eggs and oil.
- For other foods, follow feeding recommendations for the child's age and give extra two meals for 4 weeks.

ASH MOQAWI

Make flour of 1 glass rice, 1 glass corn, beans, after that roast each item separately. Mix half glass of roasted rice flour, half glass of roasted corn flour and half glass roasted wheat flour. Cook half glass of this mixture with one and half glass water for 15 minutes and add one table spoon oil.

MASH ABA:

Cook 1 and half table spoon vetch or LETY for 20 minutes, first make soup from vetch, then add half glass of wheat or corn flour and add one tablespoon oil and half glass water to it and cook the mixture for 5 minutes.

/ FERNY

Half glass rice flour, half glass cow's or goat's milk and one glass of water to be boiled and cooked slowly for 15 minutes. Then add one table spoon oil in it.

✓ Lety:

Half glass of flour, half glass of cow or goat milk, one and half spoon sugar and one spoon oil to be mixed, after that cook it and then break it into very small pieces. Boil one glass water with half glass milk and put the small pieces in the mixture of water and milk and mix it with spoon for two minutes.

► ***Counsel the Mother About Feeding Problems***

If the child is not being fed as described in the above recommendations, counsel the mother accordingly. In addition:



- If the mother reports difficulty with breastfeeding, assess breastfeeding. (See *YOUNG INFANT* chart.) As needed, show the mother correct positioning and attachment for breastfeeding.

► If the child is less than 6 months old and is taking other milk or foods:

- Build mother's confidence that she can produce all the breastmilk that the child needs.
- Suggest giving more frequent, longer breastfeeds day or night, and gradually reducing other milk or foods.

If other milk needs to be continued, counsel the mother to:

- Breastfeed as much as possible, including at night.
- Make sure that other milk is a locally appropriate breastmilk substitute.
- Make sure other milk is correctly and hygienically prepared and given in adequate amounts.
- Finish prepared milk within an hour.



► If the mother is using a bottle to feed the child:

- Recommend substituting a cup for bottle.
- Show the mother how to feed the child with a cup.

► If the child is not being fed actively, counsel the mother to:

- Sit with the child and encourage eating.
- Give the child an adequate serving in a separate plate or bowl.

► If the child is not feeding well during illness, counsel the mother to:

- Breastfeed more frequently and for longer if possible.
- Use soft, varied, appetizing, favourite foods to encourage the child to eat as much as possible, and offer frequent small feedings.
- Clear a blocked nose if it interferes with feeding.
 - Expect that appetite will improve as child gets better.
 - Follow-up any feeding problem in 5 days.



FLUID

► Advise the Mother to Increase Fluid During Illness

FOR ANY SICK CHILD:

- Breastfeed more frequently and for longer at each feed.
- Increase fluid. For example, give soup, rice water, yoghurt drinks or clean water.

FOR CHILD WITH DIARRHOEA:

- Giving extra fluid can be lifesaving. Give fluid according to Plan A or Plan B on *TREAT THE CHILD* chart.

WHEN TO RETURN

► Advise the Mother When to Return to Health Worker

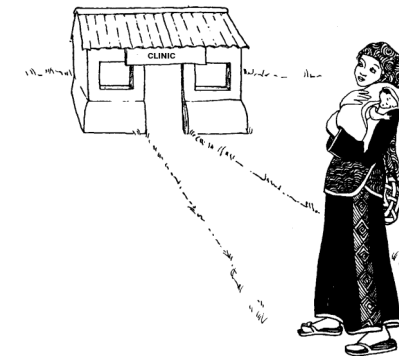
FOLLOW-UP VISIT

Advise the mother to come for follow-up at the earliest time listed for the child's problems.

If the child has:	Return for follow-up in:
PNEUMONIA DYSENTERY MALARIA, if fever persists FEVER-MALARIA UNLIKELY, if fever persists MEASLES WITH EYE OR MOUTH COMPLICATIONS	2 days
PERSISTENT DIARRHOEA ACUTE EAR INFECTION CHRONIC EAR INFECTION FEEDING PROBLEM ANY OTHER ILLNESS, if not improving	5 days
PALLOR	14 days
VERY LOW WEIGHT FOR AGE.	30 days

NEXT WELL-CHILD VISIT

Advise mother when to return for next immunization according to immunization schedule.



WHEN TO RETURN IMMEDIATELY

Advise mother to return immediately if the child has any of these signs:

Any sick child	• Not able to drink or breastfeed • Becomes sicker • Develops a fever
If child has NO PNEUMONIA: COUGH OR COLD, also return if:	• Fast breathing • Difficult breathing
If child has Diarrhoea, also return if:	• Blood in stool • Drinking poorly

► ***Counsel the Mother About Her Own Health***

- If the mother is sick, provide care for her, or refer her for help.
- If she has a breast problem (such as engorgement, sore nipples, breast infection), provide care for her or refer her for help.
- Advise her to eat well to keep up her own strength and health.
- Check the mother's immunization status and give her tetanus toxoid if needed.
- Make sure she has access to:
 - Family planning
 - Counselling on STD and AIDS prevention



ASSESS, CLASSIFY AND TREAT THE SICK YOUNG INFANT FROM BIRTH UP TO 2 MONTHS

ASSESS

ASK THE MOTHER WHAT THE YOUNG INFANT'S PROBLEMS ARE

- Determine if this is an initial or follow-up visit for this problem.
 - if follow-up visit, use the follow-up instructions on the bottom of this chart.
 - if initial visit, assess the young infant as follows:

CLASSIFY

USE ALL BOXES THAT MATCH INFANT'S
SYMPTOMS AND PROBLEMS TO
CLASSIFY THE ILLNESS.

IDENTIFY TREATMENT

CHECK FOR POSSIBLE BACTERIAL INFECTION

- ASK: Has infant had convulsions
- ASK: Is The infant vomiting everything
- LOOK: Is infant is convulsing now
- ASK/LOOK: Is infant unable to suck
- LOOK: More than 60 breaths in one minute
–Repeat count if more than 60
- LOOK for severe chest indrawing.
- LISTEN for wheeze.
- FEEL for bulging fontanelle.
- LOOK for pus draining from the ear
- LOOK for pus draining from red, swollen eyes
- LOOK for redness or pus draining from the umbilicus
–Does the redness extend to the skin?
- Measure temperature (or feel for fever or low body temperature)
- LOOK for skin pustules. Are there many or severe pustules?
- LOOK: Does the infant move only if stimulated
- LOOK: Is the infant unconscious

YOUNG
INFANT
MUST BE
CALM

**Classify
ALL
YOUNG
INFANTS**

CHECK FOR SIGNIFICANT JAUNDICE

- ASK if Jaundice started in first 24 hours of life?
- ASK age
- LOOK for Jaundice :
 - Is it severe jaundice seen in the sclera?
 - Is it extending to palms and/or soles?

**Classify
JAUNDICE**

SIGNS

CLASSIFY AS

TREATMENT

- Unable to suck or poor suck or
- Convulsions or
- Fast breathing (60 breaths per minute or more) or
- Severe chest indrawing or
- Wheeze or
- Bulging fontanelle or
- Pus draining from ear or
- Pus draining from red & swollen eyes or
- Umbilical redness extending to the skin or
- Fever (37.5°C* or above or feels hot) or low body temperature (less than 35.5°C* or feels cold) or
- Many or severe skin pustules or
- Unconscious or
- Moving only when stimulated

**POSSIBLE
SERIOUS
BACTERIAL
INFECTION**

- ▶ **Treat convulsions if present now**
- ▶ **Give first dose of intramuscular antibiotics.**
- ▶ **Treat to prevent low blood sugar.**
- ▶ **Advise mother how to keep the infant warm on the way to the hospital.**
- ▶ **Refer URGENTLY to hospital.****

- Red umbilicus or draining pus OR
- Some skin pustules OR
- Pus draining from eyes (no swelling; no redness)

**LOCAL
BACTERIAL
INFECTION**

- ▶ **Give oral amoxycillin .**
- ▶ Mother to treat local infections at home.
- ▶ Mother to give home care for young infant.
- ▶ Follow-up in 2 days.
- ▶ When to return immediately

- Jaundice started in first 24 hours of life and is still present OR
- Severe jaundice seen in sclera OR
- Jaundice extending to palms and/or soles OR
- Jaundiced Infant age 2 weeks or more

**SIGNIFICANT
JAUNDICE**

- ▶ **Encourage breastfeeding to prevent low blood sugar**
- ▶ **Advise mother how to keep the infant warm on the way to the hospital**
- ▶ **Refer URGENTLY to hospital**

- Jaundice exists on day 2 to 13 of life AND
- No jaundice on palms or soles

**NOT
SIGNIFICANT
JAUNDICE**

- ▶ Reassure mother,
- ▶ Suggest frequent breastfeeding
- ▶ Follow-up in 2 days or if danger signs
- ▶ When to return immediately

ASK: Does the young infant have diarrhoea?

- Is the infant:
Lethargic or unconscious?
Restless and irritable?
- Look for sunken eyes
- Does skin pinch of abdomen return
-Very slowly (longer than 2 seconds)?
-Slowly?
- Ask for how long
- Ask if blood is in the stool

Classify DIARRHOEA

for DEHYDRATION

Two of the following signs:

- Lethargic or unconscious
- Sunken eyes
- Skin pinch goes back very slowly.

SEVERE DEHYDRATION

- ▶ If infant does not have POSSIBLE SERIOUS BACTERIAL INFECTION:
- Give fluid for severe dehydration (Plan C).
OR
- ▶ If infant also has POSSIBLE SERIOUS BACTERIAL INFECTION:
- Refer URGENTLY to hospital with mother giving frequent sips of ORS on the way. Advise mother to continue breastfeeding.

Two of the following signs:

- Restless, irritable
- Sunken eyes
- Skin pinch goes back slowly.

SOME DEHYDRATION

- ▶ Give fluid and food for some dehydration (Plan B).
- ▶ If infant also has POSSIBLE SERIOUS BACTERIAL INFECTION:
- Refer URGENTLY to hospital with mother giving frequent sips of ORS on the way. Advise mother to continue breastfeeding.

- Not enough signs to classify as dehydration

NO DEHYDRATION

- ▶ Give fluids to treat diarrhoea at home (Plan A).

If diarrhoea 14 days or more

- Diarrhoea lasting 14 days or more

SEVERE PERSISTENT DIARRHOEA

- ▶ If the young infant is dehydrated, treat dehydration before referral unless the infant has also POSSIBLE SERIOUS BACTERIAL INFECTION.
- ▶ Refer to hospital.

If blood in stool

- Blood in the stool

BLOOD IN STOOL

- ▶ Advise the mother to keep the young infant worm on the way to hospital.
- ▶ Refer urgently to hospital.

* These thresholds are based on axillary temperature. The thresholds for rectal temperature readings are approximately 0.5°C higher.

** If referral is not possible, see *Integrated Management of Childhood Illness*, Treat the Child, Annex: "Where Referral Is Not Possible."

CHECK FOR FEEDING PROBLEM OR LOW WEIGHT IF NO INDICATION EXISTS FOR URGENT REFERRAL TO HOSPITAL:

ASK:

- Is there difficulty feeding?
- Is the infant breastfed? If yes, how many times in 24 hours?
- Does the infant usually receive any other foods or drinks? If yes, how often?
- What do you use to feed the infant?

LOOK, LISTEN, FEEL:

- Determine weight for age
- For newborn, determine birth weight
- Look for ulcers or white patches in mouth

Classify FEEDING

ASSESS BREASTFEEDING IF AN INFANT:

**Has any difficulty feeding,
Is breastfeeding less than 8 times in 24 hours,
Is taking any other foods or drinks, or
Is low weight for age,**

- Has the infant breastfed in the previous hour?

If the infant has not fed in the previous hour, ask the mother to put her infant to the breast. Observe the breastfeed for 4 minutes. (If the infant was fed during the last hour, ask the mother if she can wait and tell you when the infant is willing to feed again.)

- Is the infant able to attach?
not well attached good attachment

TO CHECK ATTACHMENT, LOOK FOR:

- Chin touching breast
- Mouth wide open
- Lower lip turned outward
- More areola visible above than below the mouth

(All of these signs should be present if the attachment is good.)

- Is the infant suckling effectively (that is, slow deep sucks, sometimes pausing)?

not suckling at all not suckling effectively suckling effectively

Clear a blocked nose if it interferes with breastfeeding.

- Look for ulcers or white patches in the mouth (thrush).

- Not well attached to breast or
- Not suckling effectively or
- Less than 8 breastfeeds in 24 hours or
- Receives other foods or drinks or
- Low weight for age or
- Thrush (ulcers or white patches in mouth)

FEEDING PROBLEM OR LOW WEIGHT

- ▶ Advise the mother to breastfeed as often and for as long as the infant wants, day and night.
 - If not well attached or not suckling effectively, teach correct positioning and attachment.
 - If breastfeeding less than 8 times in 24 hours, advise to increase frequency of feeding.
- ▶ If receiving other foods or drinks, counsel mother about breastfeeding more, reducing other foods or drinks, and using a cup.
 - If not breastfeeding at all:
 - Refer for breastfeeding counselling and possible relactation.
 - Advise about correctly preparing breastmilk substitutes and using a cup.
- ▶ If thrush, teach the mother to treat thrush at home.
- ▶ Advise mother to give home care for the young infant.
- ▶ Follow-up any feeding problem or thrush in 2 days.
Follow-up low weight for age in 14 days.

- Not low weight for age and no other signs of inadequate feeding.

NO FEEDING PROBLEM

- ▶ Advise mother to give home care for the young infant.
- ▶ Praise the mother for feeding the infant well.

THEN CHECK THE YOUNG INFANT'S IMMUNIZATION STATUS:

IMMUNIZATION SCHEDULE:

AGE

Birth
6 weeks

VACCINE

BCG OPV-0
Penta-1 OPV-1

ASSESS OTHER PROBLEMS

TREAT THE YOUNG INFANT AND COUNSEL THE MOTHER

► Give an Appropriate Oral Antibiotic For local bacterial infection:

First-line antibiotic : _____ AMOXICILLIN _____
 Second-line antibiotic: _____ COTRIMOXAZOLE _____

* Avoid cotrimoxazole in infants less than 1 month of age who are premature or jaundiced.

AGE or WEIGHT	COTRIMOXAZOLE (trimethoprim + sulphamethoxazole) ↳ Give two times daily for 5 days			AMOXICILLIN ↳ Give three times daily for 5 days	
	Adult Tablet single strength (80 mg trimethoprim + 400 mg sulphamethoxazole)	Pediatric Tablet (20 mg trimethoprim +100 mg sulphamethoxazole)	Syrup (40 mg trimethoprim +200 mg sulphamethoxazole)	Tablet 250 mg	Syrup 125 mg in 5 ml
Birth up to 1 month (< 3 kg)		1/2*	1.25 ml*		1.25 ml
1 month up to 2 months (3-4 kg)	1/4	1	2.5 ml	1/4	2.5 ml

► Give First Dose of Intramuscular Antibiotics

► Give first dose of both ampicillin and gentamicin intramuscular.

WEIGHT	GENTAMICIN Dose: 2.5 mg per kg		Ampicillin Dose: 50 mg per kg	
	Add 3 ml sterile water to 1 ml vial containing 40 mg = 4 ml at 10 mg/ml	OR Add 6 ml sterile water to 2 ml vial containing 80 mg* = 8 ml at 10 mg/ml	To a vial of 500 mg :	Add 4.5 ml sterile water = 5.0 ml at 100 mg/ml
1 kg		0.25 ml*		0.5 ml
2 kg		0.50 ml*		1.0 ml
3 kg		0.75 ml*		1.5 ml
4 kg		1.00 ml*		2.0 ml
5 kg		1.25 ml*		2.5 ml

Avoid using undiluted 40 mg/ml gentamicin. The dose is 1/4 of that listed.

► Referral is the best option for a young infant classified with POSSIBLE SERIOUS BACTERIAL INFECTION. If referral is not possible, give ampicillin and gentamicin for at least 5 days. Give ampicillin every 6 hours plus gentamicin every 8 hours. For infants in the first week of life, give gentamicin every 12

TREAT THE YOUNG INFANT AND COUNSEL THE MOTHER

► **To TREAT CONVULSIONS, See TREAT THE CHILD Chart**

► ***To Treat Diarrhoea, See TREAT THE CHILD Chart***

► ***Immunize Every Sick Young Infant, if Needed***

► ***Teach the Mother to Treat Local Infections at Home***

- Explain how the treatment is given.
- Watch her as she does the first treatment in the clinic.
- Tell her to do the treatment twice daily. She should return to the clinic if the infection worsens.

Treat Skin Pustules or Umbilical Infection

Mother should:

- Wash her hands
- Gently wash off pus and crusts with soap and water
- Dry the area
- Paint with gentian violet
- Wash her hands

Treat Thrush (ulcers or white patches in mouth)

Mother should:

- Wash her hands
- Wash mouth with clean soft cloth wrapped around the finger and wet with salt water
- Paint the mouth with half-strength gentian violet
- Wash her hands

Treat Eye Infection:

Mother should:

- Wash her hands
- Gently remove pus from eyes with clean cloth
- Apply tetracycline eye ointment in both eyes on inside of lower lid
- Wash her hands
- Treat until redness is gone

TREAT THE YOUNG INFANT AND COUNSEL THE MOTHER

► **Teach Correct Positioning and Attachment for Breastfeeding**

- Show the mother how to hold her infant



- Show her how to help the infant attach.
 - touch her nipple to the infant's lips
 - when infant's mouth is open wide move infant quickly onto her breast, infant's lower lip well below the nipple

- Try again if attachment or suckling are not good

► **Advise Mother to Give Home Care for the Young Infant**

- Breastfeed frequently as often on demand, day or night whether infant is sick or well
- Make sure the infant stays warm at all times
- **Follow-up Visit**

If the infant has:	Return for follow-up in:
LOCAL BACTERIAL INFECTION ANY FEEDING PROBLEM THRUSH	2 days
LOW WEIGHT FOR HEIGHT OR LBW	14 days
NOT SIGNIFICANT JAUNDICE	2 day

- **When to Return Immediately:**

Advise the mother to return immediately if the young infant has any of these signs:	
Breastfeeding or drinking poorly	Fast breathing
Difficult breathing	Blood in stool
Becomes sicker	Develops fever

GIVE FOLLOW-UP CARE FOR THE SICK YOUNG INFANT

► **FEEDING PROBLEM**

After 2 days: Reassess feeding: See “Check for Feeding Problem or Low Weight”

- Ask about specific feeding problems found on the initial visit.
- Reinforce the same feeding advice and counsel for new feeding problems. Follow-up in 2 days if you counsel the mother to make significant changes.
- REFER THE CHILD if you do not think that feeding will improve, or if the young infant has **lost weight**

► **LOW WEIGHT, LOW BIRTH WEIGHT**

After 14 days: Check for weight gain—weigh the infant again then reassess feeding: See “Check for Feeding Problem or Low Weight”

- ENCOURAGE THE MOTHER & CONTINUE FEEDING if the infant no longer has **low weight for age**
- ENCOURAGE THE MOTHER, CONTINUE FEEDING & CHECK WEIGHT IN 14 DAYS if the infant is **still low weight for age but is feeding well**
- COUNSEL MOTHER ABOUT FEEDING & FOLLOW UP EVERY 2 WEEKS UNTIL GAINING WEIGHT if the infant is **still low weight for age and still has a feeding problem**
- REFER TO HOSPITAL if you do not think that feeding will improve, or if the young infant has **lost weight**

GIVE FOLLOW-UP CARE FOR THE SICK YOUNG INFANT

► LOCAL BACTERIAL INFECTION

After 2 days: See if the treatment is working and to see if the child needs to be referred

For infected umbilicus and skin pustules

- REFER TO HOSPITAL if ***pus or redness remains or is worse OR if infant has signs of possible serious bacterial infection***
- CONTINUE SAME TREATMENT if ***pus and redness are improved***

For eye infection

- REFER TO HOSPITAL if ***eyes have pus and are red and swollen***
- CONTINUE SAME TREATMENT until no pus or redness if ***pus is still draining from the eye(s)*** or if eyes are improving

► BACTERIAL INFECTION UNLIKELY

After 2 days: See if child develops a bacterial infection that needs treatment or referral

Do same exam as before “Check for Possible Bacterial Infection” and follow the same treatment protocol

► THRUSH

After 2 days: Reassess white patches or ulcers in mouth. Reassess feeding exam: See “Check for Feeding Problem or Low Weight”

- REFER TO HOSPITAL if ***thrush is worse***, or the infant has ***problems with attachment or suckling***
- If ***thrush is the same or better***, and if the infant is ***feeding well***, continue half-strength gentian violet for a total of 5 days.

Name: _____ Age: _____ Weight: _____ kg Temperature: _____ °C

What are the child's problems? _____ Initial visit? ____ Follow-up Visit? ____ Date: _____

ASSESS	CLASSIFY
GENERAL DANGER SIGNS (circle signs present) Yes ____ No ____ NOT ABLE TO DRINK OR BREASTFEED VOMITS EVERYTHING HISTORY OF CONVULSION LETHARGIC OR UNCONSCIOUS CONVULSING NOW <i>If General Danger Sign present, urgently ask about cough, fever, sore throat & look for severe malnutrition and if diarrhea (assess for dehydration, hydrate) – give first dose of appropriate medication and refer urgently</i>	
COUGH OR DIFFICULT BREATHING? Yes ____ No ____ For how many days..... Count the breaths in one minute. _____ per minute, fast Breathing ? Chest indrawing Stridor Wheeze (If wheeze, go to treatment of wheezing)	
DIARRHOEA? Yes ____ No ____ For how long? ____ days Not able to drink or drinks poorly Sunken eyes Skin pinch goes back <u>Slowly</u> or <u>very slowly</u> Blood in Stool • Child is irritable or restless • Lethargic or unconscious • Drinks eagerly /thirsty	
FEVER? (by history/feels hot/temperature 37.5°C or above) Yes ____ No ____ Stiff neck Measles in last 3 months Measles now (rash + runny nose or red eyes or cough) - Clouded cornea - Mouth ulcers/deep mouth ulcers - Pus draining from eye infection High malaria risk Low malaria risk (check runny nose, ear infection, sorethroat)	
EAR PROBLEM Yes ____ No ____ Ear pain Pus draining from ear If Yes, for how long? ____ Days Tender swelling behind ear	
THROAT PROBLEM Yes ____ No ____ Throat pain Red throat and exudate on tonsils If Yes, for how long? ____ Days Swollen lymph nodes	
MALNUTRITION Yes ____ No ____ Severe visible wasting Edema of both feet Very low Weight for Age	
ANEMIA Yes ____ No ____ Palmar pallor (severe or some)	
CHECK CHILD'S IMMUNIZATION & VIT A SUPPLEMENTATION Circle immunizations/Vitamin A needed today. BCG Penta-1 Penta-2 Penta-3 Measles Vit A OPV 0 OPV 1 OPV 2 OPV 3 OPV4 Mebendazole	
ASSESS CHILD'S FEEDING if child has ANAEMIA OR LOW WEIGHT or is less than 2 years old. Do you breastfeed your child? Yes ____ No ____ If Yes, how many times in 24 hours? ____ times If Yes, do you breastfeed during the night? Yes ____ No ____ Does the child take any other food or fluids? Yes ____ No ____ If Yes, what food or fluids? How many times per day? ____ times What do you use to feed the child? Does the child receive own serving? Yes ____ No ____ Who feeds the child and how? During the illness, has the child's feeding changed? Yes ____ No ____ If Yes, how?	

TREAT (✓ all that apply and write dose of medication)				
Medication	Dose	Refer	Counsel	Follow-up
☐ Diazepam and keep the airway open ☐ Ampicillin + gentamycin ☐ Quinine ☐ Vitamin A ☐ Paracetamol		☐ Refer URGENTLY to hospital ☐ Refer for assessment	☐ Encourage breastfeeding to prevent low blood sugar ☐ Counsel mother about diet ☐ Increase fluids ☐ Continue feeding	☐ Follow-up in ____ days Return immediately if ☐ Not able to drink or breast-feed ☐ Becomes sicker ☐ Develops a fever ☐ Fast Breathing ☐ Difficult breathing ☐ Blood in stool ☐ Drinking poorly
☐ Rapid acting bronchodilator ☐ Salbutamol			☐ Soothe throat with safe remedy ☐ Dry the ear by wicking	
☐ Give Antibiotic (Co-trimoxazole, Ampicillin, Erythromycin, Ciprofloxacin, Metronidazole) ☐ Antimalarial X 3 days (Chloroquine + Sulfadoxine-pyrimethamine) ☐ Mebendazole			☐ Return for next immunization in ____ days	
☐ Rehydrate child (Plan A, Plan B or Plan C) ☐ Zinc				
☐ Apply tetracycline eye ointment if pus in eyes ☐ Gentian violet if mouth ulcers				
☐ Iron ☐ Folate				

ASSESS OTHER PROBLEMS:

MANAGEMENT OF THE SICK YOUNG INFANT AGE UP TO 2 MONTHS

Name: _____ Age: _____ Weight: _____ kg Temperature: _____ °C Date: _____

ASK: What are the infant's problems? _____ Initial visit? _____ Follow-up Visit? _____
ASSESS (Circle all signs present) **CLASSIFY**

CHECK FOR POSSIBLE SERIOUS BACTERIAL INFECTION / JAUNDICE

- Has the infant had convulsions?
- Is the infant vomiting everything?
- Is the infant convulsing now?
- Is infant unable to suck
- Look for Jaundice
 - Is severe jaundice seen in the sclera?
 - Is it extending to palms and soles?
 - Ask if jaundice started in first 24 hours?
- Listen for wheeze
- Look for pus draining from the ear
- Look for pus draining from red, swollen eyes

- Count the breaths in one minute. _____ breaths per minute
Repeat if elevated _____ Fast breathing?
- Look for severe chest indrawing
- Look and feel for bulging fontanelle.
- Look for nasal flaring
- Look at the umbilicus. Is it red or draining pus?
- Look for skin pustules. Are there 10 or more pustules or a big boil?
- Measure axillary temperature (if not possible, feel for fever. or low body temperature):
- 37.5°C or more (or feels hot)?
- Less than 35.5°C?
- See if young infant is lethargic or unconscious
- Look at young infant's movements. Less than normal?

DOES THE YOUNG INFANT HAVE DIARRHOEA?

- For how long? _____ Days Yes ___ No ___
- Is there blood in the stool?

- Look at the young infant's general condition. Is the infant:
- Lethargic or unconscious?
- Restless and irritable?
- Look for sunken eyes.
- Pinch the skin of the abdomen. Does it go back:
- Very slowly (longer than 2 seconds) - Slowly?

THEN CHECK FOR FEEDING PROBLEM & MALNUTRITION

- Is there any difficulty feeding? Yes ___ No ___ • Determine weight for age. Very low ___ Low ___ Not Low ___
- Is the infant breastfed? Yes ___ No ___
- If Yes, how many times in 24 hours? _____ times
- Does the infant usually receive any other foods or drinks? Yes ___ No ___
If Yes, how often?
- What do you use to feed the infant?
- If the infant has any difficulty feeding, is feeding less than 8 times in 24 hours, is taking any other food or drinks, or is low weight for age AND has no indications to refer urgently to hospital:**

ASSESS BREASTFEEDING:

- Has the infant breastfed in the previous hour?
If infant has not fed in the previous hour, ask the mother to put her infant to the breast. Observe the breastfeed for 4 minutes.
 - Is the infant able to attach? To check attachment, look for:
 - Chin touching breast Yes ___ No ___
 - Mouth wide open Yes ___ No ___
 - Lower lip turned outward Yes ___ No ___
 - More areola above than below the mouth Yes ___ No ___
 - no attachment at all not well attached good attachment*
 - Is the infant suckling effectively (that is, slow deep sucks, sometimes pausing)?
not suckling at all not suckling effectively suckling effectively
 - Does the mother have pain while breastfeeding?
 - Look for ulcers or white patches in the mouth (thrush).
If yes, then look for:
 - Flat or inverted nipples, or sore nipples
 - Engorged breasts or breast abscess

CHECK THE YOUNG INFANT'S IMMUNIZATION STATUS Circle immunizations needed today.

BCG	DPT 1	OPV 0	OPV 1	HEP-B 1	Return for next immunizat on: _____ Date _____
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TREAT (if all that apply and write dose of medication)

Medication	Dose	Refer	Counsel	Follow-up
Diazepam and keep the airway open		Refer	URGENTLY to _____	Encourage breastfeeding to _____ days
Ampicillin + gentamycin		hospital	prevent low blood sugar	Return immediately if _____
Paracetamol		Refer for assessment	- Teach correct positioning	Breast feeding or drinking poorly
Rehydrate child (Plan A, Plan B or Plan C)			- Home care for young infant	Difficult breathing
			Keep the young infant warm	Becomes sicker
Rapid acting bronchodilator				Fast breathing
Give Antibiotic (Amoxycillin, Co-trimoxazole, n)				Blood in stool
chloroquin+ pyramethaniline sulphadoxone				Develop Fever
Apply tetracycline eye ntment				
1 f pus in eyes				
Gentian violet if umbilical infection				

Assess other problems:

Malaria Risk Areas By Provinces

High Malaria Risk		Low Malaria Risk
Takhar		Badakhshan
Baghlan		Herat
Kunduz		Ghor
Faryab		Baghdhis
Nangahar		Farah
Kunar		Balkh
Laghman		Samangan
		Jawzan
		Kabul
		Kapisa
		Parwan
		Wardak
		Logar
		Bamyan
		Ghazni
		Paktya
		Paktika
		Kandahar
		Helmand
		Oruzgan
		Zabul
		Nimroz

WEIGHT FOR AGE CHART

